

Adult Disease Prevention Part 1 — Cancer Screening & Immunizations

Average-risk adults. Adapted from TeachIM.org (CC BY-NC 4.0); USPSTF/ACIP 2024–2025 verified. Educational use — clinician-reviewed.

When to screen

Disease: common · asymptomatic detectable window · early tx > late. **Test:** accurate · low-cost · easy · low-harm. **Benefits > harms/cost.**

Weigh comorbidities, life expectancy, and whether you'd act on a positive. Five domains: Cancer, ID, CV, Psychosocial, Family/ Safety. Use the USPSTF tool.

The 5 screenable cancers

Cancer	Who / interval	Stop
Breast	F 40–74; mammogram q2y (B)	>74 or LE<10y
Colon	45–79; colonoscopy q10y, FIT q1y, FIT-DNA q3y, CTC/sig q5y	selective 75–85
Cervical	21–29 cytology q3y; 30–65 primary HPV q5y (pref)	>65 if adequate; post-hyst
Lung	50–80, ≥20py, quit <15y; annual LDCT (B)	quit >15y / LE limited
Prostate	M 55–69, ±PSA q1–2y (SDM, C)	>70

Colonoscopy = gold standard (any + test → scope). No clinical/ self breast exam; no DRE for prostate. **Lung: Medicare needs documented SDM visit.**

Cervical — 2025 USPSTF

30–65: **primary hrHPV q5y preferred; self-collected HPV now endorsed** (clinician- OR patient-collected). Alt: cytology q3y or co-test q5y. (ACS starts 25; equivalence/interval debated.)

Immunizations

Vaccine	Who
Influenza	All, annually
COVID-19	All, ≥ each fall
Tdap	Once → Td/Tdap q10y
PCV21	≥50 (or 19–49 + chronic dz); else PCV15 → PPSV23
Zoster (RZV)	≥50, 2 doses 2–6 mo
RSV	≥75 (60–74 high-risk) SDM
Hep B	19–59 universal (≥60 if risk)
HPV	19–26 (up to 45 SDM)

Infection screening

Infection	Screen whom
HIV	Once, all 15–65
Hepatitis C	Once, all 18–79
GC / Chlamydia	Sexually active F <24; older F w/ risk
Syphilis	MSM, HIV, incarceration, sex work
Hep B	High-prevalence origin, MSM, HIV, IVDU
Latent TB	High-TB origin, congregate settings

Lifestyle risk reduction

Avoid tobacco, limit alcohol, healthy weight/diet, physical activity, sun protection, STI prevention.